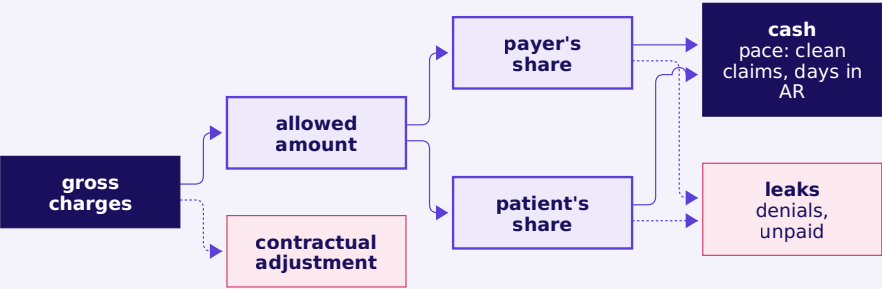


1 How does a test's charge become cash?



Allowed = payer's share + patient's share. James's tests: \$135 charged, \$64.80 written off under contract, \$70.20 allowed, \$56.16 from the plan, \$14.04 from James.

Crux. Place every revenue-cycle number on this tree, as a charge, an allowed dollar, a leak or the pace of cash, before explaining a change in it.

2 How is each revenue-cycle number worked out?

Metric	Formula
Gross charges	Tests billed x list price
Allowed amount	Payer's share + patient responsibility
Contractual adjustment	Gross charges less allowed
Net revenue	Charges less contractual adjustments less what will never be collected
Patient responsibility	Deductible + coinsurance + copay
Initial denial rate	Denied at first answer / submitted
Clean claim rate	Passed every edit untouched / entered
Days in AR	Receivable / net revenue per day
Net collection rate	Payments / allowed, once the window closes
Turnaround time	Draw to released result, median

Crux. Say the divisor, and whether it counts claims or dollars.

3 Which checks stop a lab's number from lying?

Trap	The check
A price-list rise read as growth	Track allowed dollars
Last year's allowed share, this year's claims	Check this year's remittances
Denials by count heard as dollars	State the basis
Days in AR cut by write-offs	Read write-offs beside it
Collections read before the window closes	Wait for the window

Crux. Run each check before the number leaves the team.

4 Which twenty words does a revenue-cycle meeting assume?

Word	What it means
Payer	Whoever pays the claim
Medicare	Federal cover, mostly 65 and over
Medicaid	State cover for low incomes
Requisition	The doctor's order for tests
Accession number	Barcode tying sample to order
Panel	Tests ordered by one name
Chargemaster	The lab's own price list
Deductible	Paid by the patient each plan year before the plan pays
Coinsurance	The patient's percentage after the deductible
CPT	The AMA's procedure codes
ICD-10-CM	Diagnosis codes: why it was ordered
837, 835	The claim, and the remittance
Clearinghouse	Checks and routes claims
Rejection	Bounced before the payer saw it
Denial	The payer's refusal to pay
CARC	The 835 code saying why a dollar was adjusted
Prior authorisation	Payer approval before a service
Medical necessity	The payer's test that the diagnosis justifies it
Timely filing limit	The deadline to submit a claim
PHI	Identifiable health data a plan, provider, clearinghouse or their associate holds

5 Which rules bind a US lab's data team?

Rule	What it requires
HIPAA Privacy	Use or share PHI only as the rule permits or the patient authorises
Minimum necessary	Only what the purpose needs
De-identification	Expert finding, or 18 identifiers out and no actual knowledge the rest identifies anyone
Business associate	Bound by its agreement, and directly by parts of HIPAA
Breach notice	Without unreasonable delay; 60 days at most
Medical necessity	Medicare pays only for needed tests; give the ABN when it may refuse
Timely filing	Medicare: one year from service
Offshore	No HIPAA border; contracts decide

6 Where does \$100 of a lab's charges go?

Line, illustrative	Less, \$	Left, \$
Gross charges		100
Contractual adjustments	55	45 allowed
Denials upheld, balances unpaid	3	42 net revenue
Cost of the tests	28	14 gross profit
Billing, sales, technology, admin	8	6 operating income

Crux. A \$1 leak from the \$45 allowed is 2.2 percent of it and a sixth of the lab's operating income.